

 Utah Department of Health & Human Services Licensing & Background Checks		Inspection Checklist				This tool is a preliminary inspection checklist designed to ensure consistency across all inspections. <i>(Revised Jan 2026)</i>	
Provider Name:	Elevations RTC	Facility ID:	F22-93443	Phone Number:	(801) 773-0200	Notes	
Site Name or Address:	2650 W 2700 S Syracuse, UT, 84075			Email Address:	jwilde@elevationsrtc.com; rmortensen@elevationsrtc.com; reporting@elevationsrtc.com		
Approved Capacity:	90	# of Present Residents\Clients:	42				
Please review the following items prior to the inspection: (Mark with a check mark if completed and make any necessary notes)				Please review the following items during the inspection: (Mark with a check mark if completed and make any necessary notes)			
☒	Current backgrounds in DACS			☒	Any active rule variances	No active variances.	
☒	Current staff roster collected			☒	Introduce yourself and any DHHS staff		
☒	Any license restrictions or conditions	No restrictions or conditions		☒	Staff Interviews		
☒	Any needed rule variances	No needed rule variance		☒	Clients Interviews		
Inspection Information:							
- These are initial observations and do not constitute a final inspection report. The Licensor will email you this inspection checklist after the inspection is completed if requested. An official inspection report will be sent to you once this inspection has been approved by OL management.							
Inspection Type:	Announced Annual	Date:	07/07/2026	Time Started On-site:	9:15AM	Time Ended On-site:	04:00 PM
Number of Items Under Review:		0	Name of Individual Informed of this Inspection:		Jennifer Wilde, Ryan Mortensen, Judi Jacques; Eric Flores		
Licensors(s) Conducting this Inspection:				Chanesea Butler & Robert Jones		OL Staff Observing Inspection:	

Pre - Inspection Checklist						
(Revised June 2026)						
R = Reviewed. UR = Under Review. Under Review indicates evidence of potential noncompliance. NA = Not Assessed during this inspection.						
New and Renewal Licensing Procedures	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-3(1) Until a license or certificate is approved by OL, an applicant or provider may not: (a) accept any fee; (b) enter into any agreement to provide a client service; or (c) provide any client service.	☒	☐	☐	☐	☐	
R380-600-3(2) Each applicant and provider shall comply with any applicable administrative rule, statute, zoning, fire, safety, sanitation, building and licensing laws, regulations, ordinances, and codes of the city and county in that the facility or agency will be or is located.	☒	☐	☐	☐	☐	
R380-600-3(4) A provider may not permit a staff or client to threaten, verbally or physically abuse, or use violence of any kind while interacting with a representative of the department.	☒	☐	☐	☐	☐	
R380-600-3(18) Unless previously approved by OL to provide services before receiving a license or certificate for special circumstances, a provider must submit an application, any required fee, and obtain a new or a renewed license or certificate before providing any service that requires a license or certificate.	☒	☐	☐	☐	☐	
R501-14-3(1) A provider representative shall ensure that an applicant for an initial background check completes the required application fields and disclosure statements to authorize OBP's continual monitoring of the applicant's fingerprints and applicable state registries.	☒	☐	☐	☐	☐	
R501-14-4(3)(a) The provider representative shall keep the program's roster and employee information current in DACS. (b) The provider representative shall check the roster at least monthly to verify employee information and the employment of employees due for a renewal review.	☒	☐	☐	☐	☐	
R501-14-5(2)(a) The provider representative shall submit a background check application for each applicant for an initial background check no later than two weeks from the date the applicant becomes associated with the licensee, certification, or contract. (b)(i) The provider representative shall ensure an applicant is directly supervised until OBP issues a conditional or eligible clearance determination. (ii) The provider representative shall document how the applicant remains supervised for the entirety of the applicant's supervised employment term before receiving a clearance determination.	☒	☐	☐	☐	☐	
R501-14-5(5)(a) The provider representative may not allow an applicant whose background check application is denied to have any supervised or unsupervised direct access to clients unless: (i) OBP approves a subsequent application; or (ii) the denial is overturned in an administrative hearing or by the OBP director. (b) The provider representative shall ensure an applicant initiating an appeal of a denied application works under direct supervision until OBP issues a determination regarding the appeal.	☒	☐	☐	☐	☐	
Program Policies, Procedures, and Safe Practices	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-4(1) The licensee shall submit to the office, before program implementation, policies and procedures that include: (a) a description of what constitutes sex and gender abuse, discrimination, and harassment; (b) procedures for preventing and reporting abuse, discrimination, and harassment; and (c) procedures for teaching effective and professional communication with individuals of any sexual orientations and genders.	☒	☐	☐	☐	☐	
R501-1-4(2) The licensee shall develop, implement, and comply with safe practices that: (a) ensure client health and safety; (b) ensure the needs of the client population served are met; (c) ensure that none of the program practices conflict with any administrative rule or statute before implementation; and (d) inform staff of how to manage any unique circumstances regarding the specific site's physical facility, supervision, community safety, and mixing populations.	☒	☐	☐	☐	☐	
R501-1-4(3) The licensee shall submit any change to an office approved policy or curriculum to the office for approval before implementing the proposed change.	☒	☐	☐	☐	☐	

R501-1-4(4) A congregate care program licensee shall submit to the office any policies and procedures that describe behavior management, suicide prevention, restraint, or seclusion used in the program as described in Section 26B-2-123, before implementation.	☒	☐	☐	☐	☐	
R501-1-4(5) In addition to complying with Section 26B-2-123, a congregate care program licensee shall ensure that the congregate care behavior management policy and practices reflect the following: (a) a congregate care program licensee uses behavior management techniques that are trauma-informed and appropriate for the client's age, behavior, needs, developmental level, and past experiences and defer to the least restrictive method of behavior management available to control a situation; (b) a congregate care program licensee only uses behavior management techniques that emphasize de-escalation and promote self-control, self-esteem, and independence; (c) a congregate care program licensee identifies a behavior management curriculum that emphasizes de-escalation and is compliant with Section 26B-2-123; (d) only direct care staff familiar with the child and the child's needs conduct passive physical restraint; (e) restraint is only used if it does not cause undue physical discomfort, harm, or pain to the client; (f) interventions that use painful stimuli are prohibited as a general practice; (g) passive physical restraint is used only as an emergency, temporary means of physical containment to protect the consumer, other persons, or property from immediate harm; (h) restraint only continues as long as the client presents an immediate danger to self or others; (i) passive physical restraint is not used as a convenience to staff, a substitute for programming or associated with punishment in any way; (j) a client, non-direct care staff member, or other unauthorized individual does not use any form of restraint; (k) staff do not use physical work assignments or activities that inflict pain as behavior management techniques; and (l) staff are trained to ensure the following safe practices: (i) appropriate de-escalation techniques and alternatives to restraint or seclusion; (ii) thresholds for restraints; (iii) the physiological and psychological impact of restraint; (iv) appropriate monitoring of restraint episodes; (v) how to recognize the physical signs of distress, positional asphyxia, and obtaining medical assistance; (vi) how to intervene if another staff member fails to follow correct procedures when using a restraint; (vii) time limits for restraints; (viii) the process for obtaining clinical approval for continued restraints; (ix) the procedure for documenting and reporting restraints; (x) the procedure for processing restraints with clients; (xi) the procedure for following up with staff after a restraint; (xii) how staff address injuries and complaints; (xiii) department code of conduct; and (xiv) client rights.	☒	☐	☐	☐	☐	
R501-1-4(6) A congregate care program licensee shall ensure that congregate care seclusion policy and practices reflect the following: (a) seclusion is only used to ensure the immediate safety of the child or others and is terminated as soon as the risks have been mitigated, not to exceed four hours without clinical justification; (b) staff who are familiar to the child directly supervise the child during the seclusion; (c) staff supervising seclusion ensure that any potentially harmful items or objects are removed from the seclusion environment; (d) seclusion rooms measure a minimum of 75 square feet and have a minimum ceiling height of seven feet with no equipment, hardware or furnishings that obstruct staff's view of the client or present a hazard; (e) seclusion rooms have either natural or mechanical ventilation with break resistant windows and either a break resistant two-way mirror or camera that allows for observation of the entire room; (f) seclusion rooms do not have locking capability and are not located in closets, bathrooms, unfurnished areas or other areas not designated as part of residential living space; (g) bedrooms are not utilized as a seclusion room and seclusion rooms may not be utilized as bedrooms; (h) seclusion episodes are documented in detail by the staff involved in initiating and supervising the seclusion episode; (i) seclusion episodes of more than two in a 24-hour period are supported by clinical review and documentation regarding client suitability for remaining in the program; and (j) client time-out is used when addressing behavioral issues only if: (i) a client in time-out is never physically prevented from leaving the time-out area; (ii) it takes place away from the area of activity or from other clients, such as in the client's bedroom; (iii) staff monitors the client while in time-out; and (iv) the reason for and duration of time-out is documented by staff on duty when it occurs.	☒	☐	☐	☐	☐	
R501-1-4(7) A congregate care program licensee shall develop and follow a suicide prevention policy that complies with Subsection 26B-2-123(5).	☒	☐	☐	☐	☐	
R501-1-4(8) A congregate care program licensee shall ensure that the program's licensed clinical professional conducts regular reviews of client restraints, seclusions, behavioral interventions, and time outs to inform processing discussions with clients and training for direct care staff.	☒	☐	☐	☐	☐	

R501-1-4(9) (a) Before a congregate care program licensee may accept a client or send a discharging client who is transported by a youth transportation company as defined in Section 26B-2-101, the licensee shall ensure that the transport company is registered with the office. (b) A congregate care program licensee shall report private placements to the office as described in Section 26B-2-124 by completing the congregate care out of state placement survey on the office website no later than the fifth business day of each month.	☒	☐	☐	☐	☐	
Program Administrative and Direct Service Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-6(1) The licensee shall clearly identify services to the office, public, potential client, parent, or guardian regarding: (a) current and accurate contact information; (b) the complaint reporting and resolution process; (c) a description of each service provided; (d) each program requirement and expectation; (e) eligibility criteria outlining behavior, diagnosis, situation, population, and age that can be safely served, including: (i) an outline of the behaviors and presenting issues that would be reason for discharge or exclusion from the program; and (ii) a statement that the program may not take placement of a child whose needs exceed the scope or ability of the program to reasonably manage; (f) each cost, fee, and expense for a service and refund policy; and (g) identification of each non-clinical, extracurricular, or supplemental service offered or referred.	☒	☐	☐	☐	☐	
R501-1-6(2) The licensee shall post the following in conspicuous places where each visitor, staff, and client may view: (a) abuse reporting laws as described in Sections 80-2-609 and 26B-6-205; (b) civil rights notice; (c) Americans with Disabilities Act notice; (e) any office notice of agency action; (f) a client rights poster in a residential setting except in a foster home or where prohibited by Settings Final Rule; and (g) department code of conduct poster.	☒	☐	☐	☐	☐	
Program Physical Facilities and Safety	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-8(1) The licensee shall ensure: (a) the appearance & cleanliness of the building/grounds are maintained & free from health/fire hazards; (b) any appliances, plumbing, electrical, HVAC, and furnishings are maintained in operating order and in a clean and safe condition; (c) fire drills in non-outpatient programs are conducted at least quarterly and documented, including feedback regarding response time and process; (d) a phone that can be used to call 911 is always available on-site when clients are present; (e) bathroom facilities for staff and clients allow for individual privacy and afford reasonable accommodation based on gender identity; (f) each bathroom is properly equipped with toilet paper, paper towels or a dryer, and soap; (g) each bathroom is ventilated by mechanical means or equipped with a window that opens; (h) non-prescription medication, if stored on-site, is stored in original manufacturer's packaging together with the manufacturer's directions and warnings; and (i) prescription medication, if stored on-site, is stored in original pharmacy packaging or individual pharmacy bubble pack together with the pharmacy label, directions, and warnings.	☒	☐	☐	☐	☐	
R501-1-8(2) The licensee shall accommodate a client with physical disabilities as needed or appropriately refer to comparable services.	☒	☐	☐	☐	☐	

R501-1-8(3) The licensee shall maintain medication and potentially hazardous items on-site lawfully, responsibly, and with consideration of the safety and risk level of the population served to include locked storage for each medication and hazardous chemical that is not in active use.	☒	☐	☐	☐	☐	
R501-1-8(4) The licensee shall maintain a first aid kit	☒	☐	☐	☐	☐	
Residential Program Additional Facilities and Safety Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-9(1) A residential licensee shall ensure: (a) designated space is available for records, administrative work, & confidential phone calls for clients; (c) live-in staff have dedicated bedrooms & bathrooms separate from client use; (d) each bedroom designated for a client is comparable to other similarly utilized bedrooms with similar access, location, space, finishings, and furnishings; (e) clients are not locked in bedrooms; (f) a mirror or safety mirror is secured to each bathroom wall at a convenient height; (g) each bathroom is placed to allow access to each client without disturbing any other client during sleeping hours; (h) each bath or shower allows for individual privacy; (i) each client is supplied with hygiene supplies; (j) each sleeping area has a source of natural light and is ventilated by mechanical means or is equipped with a window that opens; (k) each client has a similar solid type of bed or sleeping equipment to any other client in the program; (m) there are separate containers for soiled & clean laundry, if the program provides common laundry for towels, bedding or clothing; (o) equipment and supplies for washing & drying laundry are provided, if the program permits clients to do their own laundry; (p) there is at least 60 sq ft per person in a multiple occupancy bedroom and 80 sq ft in a single occupant bedroom.	☒	☐	☐	☐	☐	
R501-1-9(3) The licensee utilizing seclusion rooms shall ensure the following: (a) seclusion rooms measure a minimum of 75 sq ft and have a minimum ceiling height of 7 ft with no equipment, hardware or furnishings that obstruct staff's view of the client or present a hazard; (b) a seclusion room shall have either natural or mechanical ventilation with break resistant windows and either a break resistant two-way mirror or camera that allows for observation of the entire room; (c) a seclusion room may not have locking capability and may not be located in closets, bathrooms, unfurnished areas or other areas not designated as part of residential living space; and (d) a bedroom may not be utilized as a seclusion room and a seclusion room may not be utilized as a bedroom.	☒	☐	☐	☐	☐	
Food Service Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-10(2) A licensee that provides meals shall: (ii) ensure that each client with special nutritional needs has food storage and a preparation area that is not exposed to any identified allergen or contaminant; and (f) provide adequate dining space for clients that is maintained in a clean and safe condition.	☒	☐	☐	☐	☐	
Program Client Record Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-11(2) The licensee shall document a plan detailing how each program staff and client file is maintained and remains available to the office and other agencies legally authorized to access the files for seven years regardless of whether the program remains licensed.	☒	☐	☐	☐	☐	
Rule Compliance, Penalties, Agency Action Reviews, and Appeals	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-8(11) Any owner identified in a license or certificate revocation action may not be approved for a license or certification of any other program or facility overseen by OL for five years from the date the revocation was made effective.	☒	☐	☐	☐	☐	

Congregate Care Residential Treatment Programs - Inspection Checklist

(Revised June 2026)

R = Reviewed.

UR = Under Review. Under Review indicates evidence of potential noncompliance.

NA = Not Assessed during this inspection.

Administration	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
26B-7-117(3) A person that is licensed by the department to provide residential treatment for a substance use disorder shall include as part of the person's admissions materials a question asking whether the individual seeking treatment has ever received services from a syringe exchange program.	☐	☐	☒	☐	☐	
R501-19-4(2) Any qualifying residential treatment provider: (a) may not deny any client services solely based on a client receiving medication assisted treatment; and (b) shall ensure any client continues the medication assisted treatment while receiving services from the provider.	☒	☐	☐	☐	☐	
R501-19-4(3) Each residential treatment provider shall ensure its policies include client privacy accommodation in each bedroom space while ensuring client health and safety	☒	☐	☐	☐	☐	
R501-19-4(4) Each residential treatment provider serving a child shall: (a) provide direct supervision that meets supervision and ratio requirements discribed in this section; (b) ensure two direct care staff are always on duty; (c) maintain a staff-to-client ratio of one staff to every four clients except: (i) as otherwise required by a Department of Health and Human Services (department) contract; or (ii) to reduce ratios to one staff to every 16 clients during client sleeping hours; (d) only decrease the number of staff as described in this section if: (i) each client is appropriately supervised to ensure health and safety at the ratio; and (ii) each direct care staff remains awake while on duty. (e) increase the staff-to-client ratio as necessary to ensure the health and safety of the current client population. (f) only allow direct care staff to perform direct supervision with line of sight check-ins every 15 minutes; (g) ensure that any direct care staff member assigned to a client's one-on-one supervision is not counted at the same time in the staffing ratio for any other client, except in an emergency situation; (h) only utilize on-site video surveillance to directly supervise a client in time out or seclusion or as an enhancement to minimum supervision ratio requirements; (i) conduct and document physical check-ins every 15-minutes when a client is being monitored by video; and (j) only use video surveillance in a bedroom: (i) with client, parent, or guardian permission; (ii) when there is a documented need; (iii) when the provider monitors any camera or physically checks in at intervals of 15 minutes or less;	☒	☐	☐	☐	☐	
R501-19-4(5) Each residential treatment provider serving a child may provide step-down privileges including authorized departures from the program and unsupervised time, if the provider: (a) maintains a staff-to-client ratio of one direct care staff to every four clients; (b) documents in the client record and communicates to each of the client's direct care staff: (i) the individualized justification for the step-down privileges; and (ii) which privileges are authorized by a clinical professional; (c) obtains written parental or guardian consent before allowing step-down privileges; and (d) provides a policy to each client and parent or guardian that includes: (i) a description of what constitutes authorized departure and unsupervised time; (ii) a description of how each step-down privilege, including authorized departure or unsupervised time, is achieved or rescinded; (iii) a statement that the provider will immediately communicate to each client parent or guardian and direct care staff when any step-down privilege has been rescinded; and (iv) a statement that no step-down client is allowed to perform any direct care staff duty.	☒	☐	☐	☐	☐	
R501-19-4(6) Each residential treatment provider shall make any necessary accommodation to allow a child to continue that child's education with a curriculum approved by the State Board of Education.	☒	☐	☐	☐	☐	

R501-19-4(7) Each residential treatment provider that offers education shall use a curriculum that is recognized by an educational accreditation organization, including the State Board of Education or the National School Accreditation Board.	☒	☐	☐	☐	☐	
R501-19-4(8) (a) In addition to the behavior management policy and training requirements listed in Rule R501-1, each residential treatment provider serving youth shall ensure each direct care staff member is trained through a nationally or regionally recognized curriculum and can recognize the difference between a restraint and an emergency safety intervention. (b) An emergency safety intervention may not be used as a behavioral compliance method. (c) The provider shall only use an emergency safety intervention to ensure immediate safety of a client or staff member. (d) An emergency safety intervention is subject to each requirement of a restraint for reporting, debriefing, clinical reviews, and training. (e) An emergency safety intervention may exceed the limitations of any restraint listed in Rule R501-1 with documented justification explaining why a regular restraint or other less intrusive intervention was not used	☒	☐	☐	☐	☐	
R501-19-4(10) Each residential treatment provider providing services to a substance use disorder client shall: (a) only admit a substance use disorder client with a level of care that falls within American Society of Addiction Medicine levels 3.1 through 3.5; and (b) obtain any required license before providing any service to a substance use disorder client outside of the residential setting with a level of care described in Subsection (10)(a), unless otherwise outlined in categorical rule.	☐	☐	☒	☐	☐	
R501-19-4(11) Each residential treatment provider that allows a client to participate in food preparation shall ensure the client is trained in safe food handling practices and the provider justifies the client's participation in writing.	☒	☐	☐	☐	☐	
R501-19-4(12) Each residential treatment provider shall provide individual, group, and family counseling or other treatment, including skills development, at least weekly or as outlined in the individual's treatment plan.	☒	☐	☐	☐	☐	
R501-19-4(13) A clinical professional shall oversee any therapeutic services conducted in the therapeutic environment including: (i) life skill development; (ii) psychoeducation; and (iii) social coaching.	☒	☐	☐	☐	☐	
R501-19-4(14) Each residential treatment provider shall document the time and date of each service provided to each client and include the signature of the individual providing the service.	☒	☐	☐	☐	☐	
R501-19-4(15) Each residential treatment provider shall provide indoor space for free time and informal client activities.	☒	☐	☐	☐	☐	
Requirements for Intermediate Secure Treatment	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-19-5(1)(a) Each intermediate secure treatment provider shall clearly define in policy the responsibilities of the manager described in Section R501-1-15. (b) The provider shall ensure the manager described in Subsection R501-1-15(2): (i) is at least 25 years old; (ii) has a bachelor's degree or equivalent training in a human service-related field; and (iii) has at least three years management experience in a residential or secure treatment setting.	☒	☐	☐	☐	☐	
R501-19-5(2)(a) Subsection R501-19-4(4)(c) does not apply to an intermediate secure treatment provider serving youth. (b) An intermediate secure treatment provider serving youth shall maintain a staff-to-client ratio of one staff to every five clients.	☒	☐	☐	☐	☐	
R501-19-5(3) Each intermediate secure treatment provider shall ensure that each direct care staff working in an intermediate secure treatment program: (a) is trained to work with a child with behavioral or mental health needs; and (b) works under the supervision of a licensed clinical professional.	☒	☐	☐	☐	☐	

R501-19-5(4) Each intermediate secure treatment provider shall ensure each direct care staff completes 30 hours of additional training annually regarding: (a) client record and incident documentation; (b) client rules; (c) human relations and communication skills; (d) maintaining staff, client, and visitor safety in a secure setting; (e) problem-solving and guidance ; (f) the special needs of children and families; and (g) universal precautions for blood-borne pathogens.	☒	☐	☐	☐	☐	
R501-19-5(5) Each intermediate secure treatment provider shall incorporate the use of fixtures and furnishings that help limit self-harm and suicide including: (a) non-exposed fire sprinkler heads; (b) plexiglass or safety glass; (c) pressure release robe hooks; (d) recessed lighting; and (e) sealed light fixtures.	☒	☐	☐	☐	☐	
Specialized Services Required to Serve Clients Under the Division of Services for People with Disabilities	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-19-6(1) Each residential treatment provider serving a Division of Services for People with Disabilities (DSPD) client shall: (a) apply for any unearned income benefits for which the client is eligible, in conjunction with the support coordinator for DSPD and each client's parent or guardian; (b) develop and adhere to policies and procedures governing the daily operation and activity available and applicable to each client or visitor; (c) ensure the facility is located within a reasonable distance from a: (i) church (ii) recreation or other community facilities and (iii) school; (d) maintain a record of income and client service fees; (e) maintain a record of each fund deposited with the residential facility for client use; (f) maintain a list of each deposit and withdrawal; (g) maintain a receipt signed by the client and professional staff for any purchase over \$20; (h) maintain a record of each client's petty cash fund; (i) present each client with an individual plan that addresses appropriate day treatment; (j) share a monthly activity schedule with each client; and (k) specify, in policy, the amount of time any non-client individual may stay as an overnight guest.	☐	☐	☒	☐	☐	
R501-19-6(2) If there is a conflict between this rule and the settings final rule, as defined in Rule R501-1, the settings final rule shall prevail.	☐	☐	☒	☐	☐	

Congregate Care Residential Treatment Programs - Inspection Checklist

(Revised June 2026)

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Administration	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
26B-7-117(3) A person that is licensed by the department to provide residential treatment for a substance use disorder shall include as part of the person's admissions materials a question asking whether the individual seeking treatment has ever received services from a syringe exchange program.	☐	☐	☒	☐	☐	
R501-19-4(2) Any qualifying residential treatment provider: (a) may not deny any client services solely based on a client receiving medication assisted treatment; and (b) shall ensure any client continues the medication assisted treatment while receiving services from the provider.	☒	☐	☐	☐	☐	
R501-19-4(3) Each residential treatment provider shall ensure its policies include client privacy accommodation in each bedroom space while ensuring client health and safety	☒	☐	☐	☐	☐	
R501-19-4(4) Each residential treatment provider serving a child shall: (a) provide direct supervision that meets supervision and ratio requirements discribed in this section; (b) ensure two direct care staff are always on duty; (c) maintain a staff-to-client ratio of one staff to every four clients except: (i) as otherwise required by a Department of Health and Human Services (department) contract; or (ii) to reduce ratios to one staff to every 16 clients during client sleeping hours; (d) only decrease the number of staff as described in this section if: (i) each client is appropriately supervised to ensure health and safety at the ratio; and (ii) each direct care staff remains awake while on duty. (e) increase the staff-to-client ratio as necessary to ensure the health and safety of the current client population. (f) only allow direct care staff to perform direct supervision with line of sight check-ins every 15 minutes; (g) ensure that any direct care staff member assigned to a client's one-on-one supervision is not counted at the same time in the staffing ratio for any other client, except in an emergency situation; (h) only utilize on-site video surveillance to directly supervise a client in time out or seclusion or as an enhancement to minimum supervision ratio requirements; (i) conduct and document physical check-ins every 15-minutes when a client is being monitored by video; and (j) only use video surveillance in a bedroom: (i) with client, parent, or guardian permission; (ii) when there is a documented need; (iii) when the provider monitors any camera or physically checks in at intervals of 15 minutes or less;	☒	☐	☐	☐	☐	
R501-19-4(5) Each residential treatment provider serving a child may provide step-down privileges including authorized departures from the program and unsupervised time, if the provider: (a) maintains a staff-to-client ratio of one direct care staff to every four clients; (b) documents in the client record and communicates to each of the client's direct care staff: (i) the individualized justification for the step-down privileges; and (ii) which privileges are authorized by a clinical professional; (c) obtains written parental or guardian consent before allowing step-down privileges; and (d) provides a policy to each client and parent or guardian that includes: (i) a description of what constitutes authorized departure and unsupervised time; (ii) a description of how each step-down privilege, including authorized departure or unsupervised time, is achieved or rescinded; (iii) a statement that the provider will immediately communicate to each client parent or guardian and direct care staff when any step-down privilege has been rescinded; and (iv) a statement that no step-down client is allowed to perform any direct care staff duty.	☒	☐	☐	☐	☐	
R501-19-4(6) Each residential treatment provider shall make any necessary accommodation to allow a child to continue that child's education with a curriculum approved by the State Board of Education.	☒	☐	☐	☐	☐	

R501-19-4(7) Each residential treatment provider that offers education shall use a curriculum that is recognized by an educational accreditation organization, including the State Board of Education or the National School Accreditation Board.	☒	☐	☐	☐	☐	
R501-19-4(8) (a) In addition to the behavior management policy and training requirements listed in Rule R501-1, each residential treatment provider serving youth shall ensure each direct care staff member is trained through a nationally or regionally recognized curriculum and can recognize the difference between a restraint and an emergency safety intervention. (b) An emergency safety intervention may not be used as a behavioral compliance method. (c) The provider shall only use an emergency safety intervention to ensure immediate safety of a client or staff member. (d) An emergency safety intervention is subject to each requirement of a restraint for reporting, debriefing, clinical reviews, and training. (e) An emergency safety intervention may exceed the limitations of any restraint listed in Rule R501-1 with documented justification explaining why a regular restraint or other less intrusive intervention was not used	☒	☐	☐	☐	☐	
R501-19-4(10) Each residential treatment provider providing services to a substance use disorder client shall: (a) only admit a substance use disorder client with a level of care that falls within American Society of Addiction Medicine levels 3.1 through 3.5; and (b) obtain any required license before providing any service to a substance use disorder client outside of the residential setting with a level of care described in Subsection (10)(a), unless otherwise outlined in categorical rule.	☐	☐	☒	☐	☐	
R501-19-4(11) Each residential treatment provider that allows a client to participate in food preparation shall ensure the client is trained in safe food handling practices and the provider justifies the client's participation in writing.	☒	☐	☐	☐	☐	
R501-19-4(12) Each residential treatment provider shall provide individual, group, and family counseling or other treatment, including skills development, at least weekly or as outlined in the individual's treatment plan.	☒	☐	☐	☐	☐	
R501-19-4(13) A clinical professional shall oversee any therapeutic services conducted in the therapeutic environment including: (i) life skill development; (ii) psychoeducation; and (iii) social coaching.	☒	☐	☐	☐	☐	
R501-19-4(14) Each residential treatment provider shall document the time and date of each service provided to each client and include the signature of the individual providing the service.	☐	☐	☒	☐	☐	
R501-19-4(15) Each residential treatment provider shall provide indoor space for free time and informal client activities.	☒	☐	☐	☐	☐	
Requirements for Intermediate Secure Treatment	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-19-5(1)(a) Each intermediate secure treatment provider shall clearly define in policy the responsibilities of the manager described in Section R501-1-15. (b) The provider shall ensure the manager described in Subsection R501-1-15(2): (i) is at least 25 years old; (ii) has a bachelor's degree or equivalent training in a human service-related field; and (iii) has at least three years management experience in a residential or secure treatment setting.	☒	☐	☐	☐	☐	
R501-19-5(2)(a) Subsection R501-19-4(4)(c) does not apply to an intermediate secure treatment provider serving youth. (b) An intermediate secure treatment provider serving youth shall maintain a staff-to-client ratio of one staff to every five clients.	☒	☐	☐	☐	☐	
R501-19-5(3) Each intermediate secure treatment provider shall ensure that each direct care staff working in an intermediate secure treatment program: (a) is trained to work with a child with behavioral or mental health needs; and (b) works under the supervision of a licensed clinical professional.	☒	☐	☐	☐	☐	

R501-19-5(4) Each intermediate secure treatment provider shall ensure each direct care staff completes 30 hours of additional training annually regarding: (a) client record and incident documentation; (b) client rules; (c) human relations and communication skills; (d) maintaining staff, client, and visitor safety in a secure setting; (e) problem-solving and guidance ; (f) the special needs of children and families; and (g) universal precautions for blood-borne pathogens.	☒	☐	☐	☐	☐	
R501-19-5(5) Each intermediate secure treatment provider shall incorporate the use of fixtures and furnishings that help limit self-harm and suicide including: (a) non-exposed fire sprinkler heads; (b) plexiglass or safety glass; (c) pressure release robe hooks; (d) recessed lighting; and (e) sealed light fixtures.	☒	☐	☐	☐	☐	
Specialized Services Required to Serve Clients Under the Division of Services for People with Disabilities	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-19-6(1) Each residential treatment provider serving a Division of Services for People with Disabilities (DSPD) client shall: (a) apply for any unearned income benefits for which the client is eligible, in conjunction with the support coordinator for DSPD and each client's parent or guardian; (b) develop and adhere to policies and procedures governing the daily operation and activity available and applicable to each client or visitor; (c) ensure the facility is located within a reasonable distance from a: (i) church (ii) recreation or other community facilities and (iii) school; (d) maintain a record of income and client service fees; (e) maintain a record of each fund deposited with the residential facility for client use; (f) maintain a list of each deposit and withdrawal; (g) maintain a receipt signed by the client and professional staff for any purchase over \$20; (h) maintain a record of each client's petty cash fund; (i) present each client with an individual plan that addresses appropriate day treatment; (j) share a monthly activity schedule with each client; and (k) specify, in policy, the amount of time any non-client individual may stay as an overnight guest.	☐	☐	☒	☐	☐	
R501-19-6(2) If there is a conflict between this rule and the settings final rule, as defined in Rule R501-1, the settings final rule shall prevail.	☐	☐	☒	☐	☐	